CHIEF COMPLAINT: Progressive non-productive dry cough and exertional breathlessness over eight months.

HISTORY OF PRESENT ILLNESS: The patient is a 66-year-old retired civil engineer with a 15 pack-year remote smoking history who presents for pulmonary consultation regarding worsening shortness of breath. He reports dyspnea when climbing a single flight of stairs or walking briskly on level ground. He denies hemoptysis, fevers, chills, paroxysmal nocturnal dyspnea, orthopnea, or wheezing. No history of occupational asbestos, silica, or grain dust exposure. Review of systems is positive for dry cough, mild fatigue, and morning joint stiffness in the hands without raynaud phenomenon or dysphagia.

PHYSICAL EXAMINATION:

Vital Signs: Blood pressure 128/76 mmHg, pulse 74 bpm regular, respiratory rate 20 breaths per minute, afebrile at 36.8 degrees Celsius. Resting oxygen saturation on room air is 93%, dropping to 86% following a two-minute 50-meter corridor walk test with rapid recovery upon rest.

HEENT: Normocephalic, atraumatic. Pupils equal and reactive. Conjunctivae clear.

Cardiovascular: Regular rate and rhythm. S1 and S2 present. Accentuated pulmonic component of S2 (P2). No murmurs or gallops. Jugular venous pressure is 3 cm above the sternal angle at 45 degrees.

Pulmonary: Symmetric chest expansion. Bilateral basal end-inspiratory dry 'Velcro-like' fine crackles (crepitations) auscultated extending to the mid-lung zones bilaterally. No wheezes, rhonchi, or pleural friction rubs.

Extremities: Symmetric digital clubbing noted in the bilateral fingers (Schamroth sign positive). No peripheral cyanosis or pedal edema.

DIAGNOSTIC WORKUP:

High-resolution computed tomography (HRCT) of the chest reveals bilateral, subpleural, basally predominant reticular opacities with architectural distortion, traction bronchiectasis, and well-defined subpleural honeycombing without significant ground-glass attenuation, meeting criteria for a definitive Usual Interstitial Pneumonia (UIP) pattern. Pulmonary function testing demonstrates a severe restrictive ventilatory defect: FVC 58% of predicted, FEV1 64% of predicted, FEV1/FVC ratio 84%, TLC 62% of predicted, and diffusing capacity for carbon monoxide (DLCO) markedly reduced at 41% of predicted adjusted for hemoglobin.

IMPRESSION AND RECOMMENDATIONS:

1. Idiopathic pulmonary fibrosis (IPF) supported by typical HRCT UIP pattern, restrictive spirometry, and profound diffusion impairment.

2. Order autoimmune serologies (ANA, RF, anti-CCP, ENA, myositis panel) to exclude connective tissue disease-associated interstitial lung disease.

3. Initiate antifibrotic therapy with oral nintedanib or pirfenidone following baseline hepatic function panel.

4. Prescribe ambulatory pulse-dose oxygen therapy at 3 liters per minute for exertional desaturation.

5. Referral for formal cardiopulmonary rehabilitation and tertiary lung transplant evaluation.